جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ناهد محمد محمد عبد المجيد

المحافظة : الجيزه **الحي / المدينة :** اوسيم **العنوان :** 18 ش اولاد عطيه - برطس مركز اوسيم

رقم البطاقة : 28402252102902 **رقم التليفون :** 01124914987 **تاريخ تسجيل الإستمارة :** 24-10-2026

العمر(تاريخ الميلاد) : 25-02-1984 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي الاورام المنتشر [14456]

قسم العلاج : الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ناهد محمد محمد عبد المجيد
National ID: 28402252102902

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pin Medication And Anti Emetics

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
ناهد
محمد محمد عبدالمجيد
١٨ ش اولاد عطيه - برطس
مركز أوسيم - الجيزه
١٩٨٤/٠
٢٨٤٠٢٢٥٢١٠٢٩٠٢
LU6701744


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٢٠٢٦/٠٦
ربة منزل
انثى
مسلمة
متزوجة
عماد سعد امام محمد
البطاقة سارية حتى ٢٠٣٣/٠٦/٢٦
١

NHTMRI

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

قسم الباثولوجي

PATHOLOGY DEPARTMENT
HISTOPATHOLOGICAL REPORT

Patient name: ناهد محمد عبدالمجيد **Age:** 42years **Sex:** female
Request type: مبادره **Sub request type:**
Hospital no: **Pathology no.:**551B-2026
Department: صحه مرأه **Date of result:**7/7/2026
Referred by: د/مروه حسن **Date of biopsy:**29/6/2026
Nature of specimen: tru-cut biopsy **I.D. Number:**28402252102902
Clinical diagnosis: Right breast mass

Gross pathology :
Multiple fragmented grayish white tissue cores up to 0.5x0.4cm long, totally submitted.

Immunohistochemical Technique:
Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against ER- PR- Her2- KI67

Microscopic picture: Section examined revealed core of mammary tissue showing invasive ductal carcinoma of no special type (NST). The tumor forms nest, sheets and cribriform pattern infiltrating among desmoplastic stroma. Tumor cells showing moderate anaplasia and moderate tendency to from tubules with frequent mitotic figures. There is no ductal carcinoma insitu is seen. No peri tumoral lymphovasculr emboli or perineural infiltration in core tissue examined.

Immunohistochemical Result
ER: Positive Strong nuclear staining in 90% of tumor cells (Allred Score 8/8).
PR: Moderate nuclear staining in 40% of tumor cells (Allred Score 6/8).
Her2: Moderate compelete membranous staining of >10% tumor cells (Score 2+)
Equivocal for SISH.
Ki67: Positive nuclear staining of 60% of tumor cells.

Diagnosis: Right breast mass, tru-cut biopsy:
Invasive duct carcinoma (NST), Grade II.

Signature:
Dr.Mona Mohamed.M.D

SISH

Pathology Lab. Manager

BASMA REFAAT M.D

Patient ID: 96550	Patient Name English: Nahed Mohamed Mohamed
Age: 42 years old	2026/06/23 11:41:53 AM :Report Date

Bilateral Breast Sono-Mammography

CLINICAL SUMMARY: right long standing painful lump.

FINDINGS:

ACR: b (scattered fibro-glandular densities)

LEFT BREAST:

- No masses, asymmetry or distortion or suspicious microcalcifications.
- Normal skin / nipple / areolar complex.

Complementary ultrasound:

- Preserved fibroglandular tissue
- No solid or cystic lesion
- Normal nipple/areola/complex.
- No pathological axillary lymphnode

RIGHT BREAST:

- **Low energy (Non contrast) images** : UOQ & central & UIQ asymmetry with distortion & overlaying scattered calcifications extend from retroareolar to posterior third of the breast measures 10x8x7cm
- Diffuse skin thickness & nipple retraction.

Complementary ultrasound:

- 8-3:00 Diffuse pathological process in form area of altered parenchyma, mount to mass formation reaching the nipple with focal area of skin tethering & diffuse skin thickness & retracted nipple.
- Level I & II more than 3 discrete pathological nodes, the largest in level measures 14x7mm & in level II 9x5mm.

IMPRESSION:

LEFT BREAST: (BIRADS1 = normal)

RIGHT BREAST: (BIRADS 5= highly suggestive of malignancy): diffuse pathological process , highly suggestive of malignancy for US guided biopsy

Best regards,

Dr. Marwa Ramzy, MD, FRCR

Name: NAHED MOHAMED MOHAMED	Date : 18/8/2026
Age : 31 TE : 01127872211	Exam MAMMO

Clinical presentation:

Right mastaliga, and swelling with skin changes, for assessment

MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND FOR BOTH BREASTS REVEALED:

- The right breast is the seat of diffuse increase density and skin thickening with thickening and coarsening architecture noted involving the four quadrants of right breast. Ultrasound examination showed large central glandular area of architectural distortion measured about 16x10 cm with vascularity seen para areolar involving most anterior central part of the breast reaching to subcutaneous region and nipple , some areas showed skin affection .
- Enlarged right axillary lymphadenopathy the largest 3 cm
- No micro calcification seen.
- Normal left mammogram.
- Normal skin thickness and contour of left breast.

Opinion :

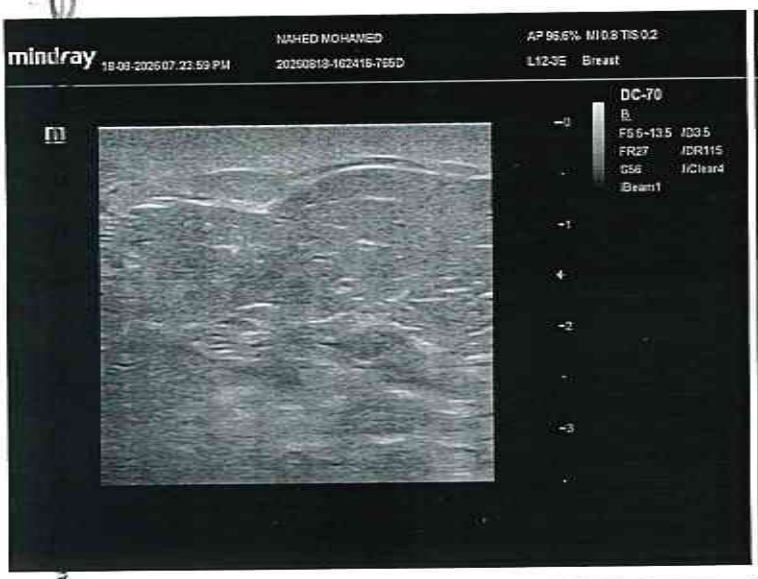
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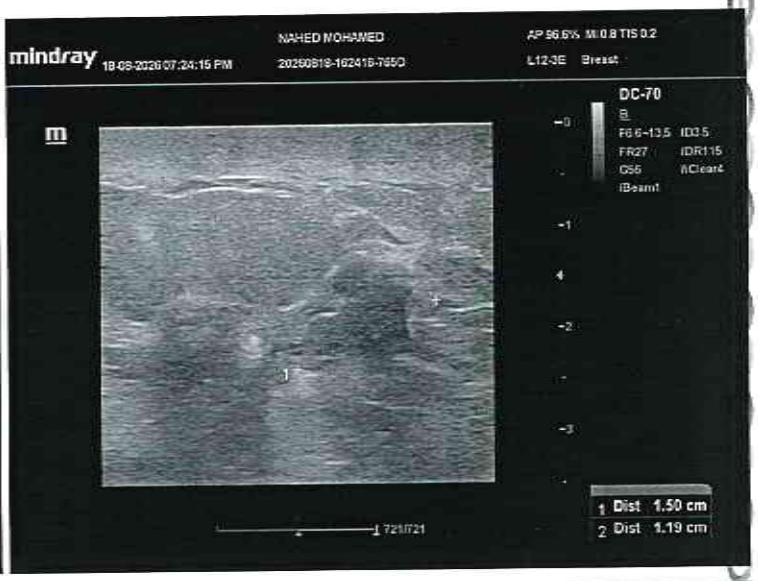
Best regards,
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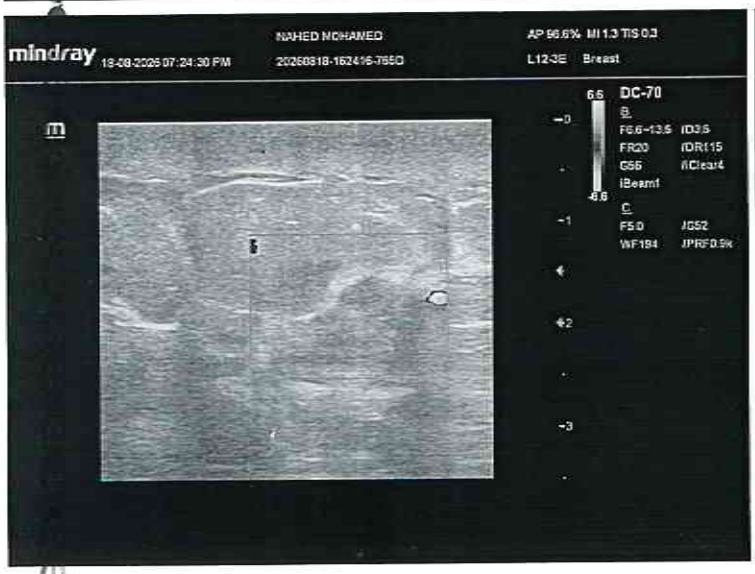
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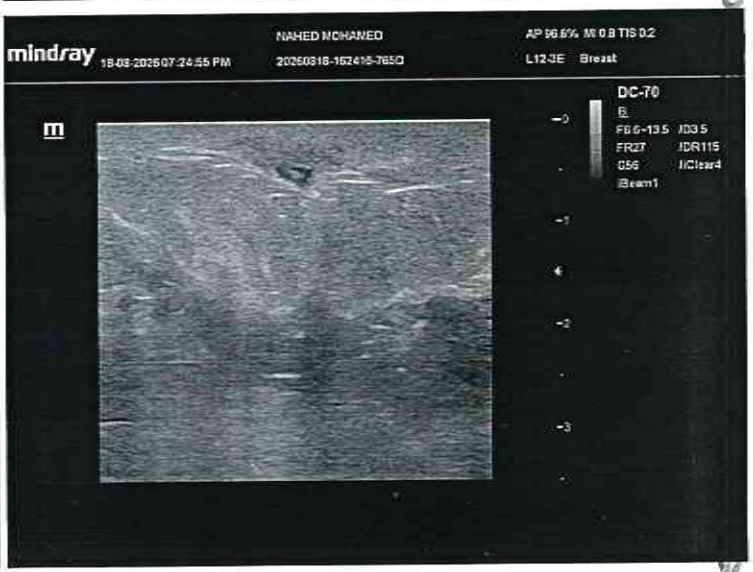
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mindray
NAHED MOHAMED
AP 96.6% MI 0.8 TIS 0.2
L12-3E Breast
DC-70


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ناهد
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NHTMRI

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المعهد القومي للكبد والجهاز الهضمي
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Best regards,

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(٤)

١٠٠ مليون صحة

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



لم يتم الرد
أكثر من

الست المصرية هتبقى صحة مصر

منظمة الصحة العالمية
جمهورية مصر العربية

الاسم: شاهد محمد محمد الحبيب التاريخ: ١٨/٥/٢٠٢٢
الرقم القومي: ٢٩٠٠٥١٠٢٠١٠٤٤
رقم التليفون: ٠١١٤٩١٤٦٨٧

مستهدفة بالتوعية

مستهدفة بالتوعية والفحص الإكلينيكي

الوزن	الطول	مؤشر كتلة الجسم	قياس السكر العشوائي	قياس ضغط
٩٥	١٦٥		٩٠	١٢/٨

٩٥/١٦٥/٢٠

* فكري تفسك باستمرار بموعد الفحص الذاتي شهريًا

أغسطس 2022		سبتمبر 2022		أكتوبر 2022	
نوفمبر 2022		ديسمبر 2022		يناير 2023	
فبراير 2023		مارس 2023		أبريل 2023	
مايو 2023		يونيو 2023		يوليو 2023	
أغسطس 2023		سبتمبر 2023		أكتوبر 2023	
نوفمبر 2023		ديسمبر 2023		يناير 2024	
فبراير 2024		مارس 2024		أبريل 2024	
مايو 2024		يونيو 2024		أبريل 2024	

- حافظي علي قياس السكر والضغط مرة كل ٦ شهور.
- إذا كنتي في سن الزواج أو الإنجاب، إستشيري بإنتظام طبيب تنظيم الأسرة.
- إطمني ، الكشف المبكر بيساعد في السيطرة على الأمراض، وزيادة نسب الشفاء.

موعد الفحص القادم: مستشفى أو مركز توقيع الطبيب:

المكان المحولة له للتقييم: موعد التقييم:

15335 www.100millionseha.eg

Name: NAHED MOHAMED MOHAMED	Date : 18/8/2026
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Opinion :

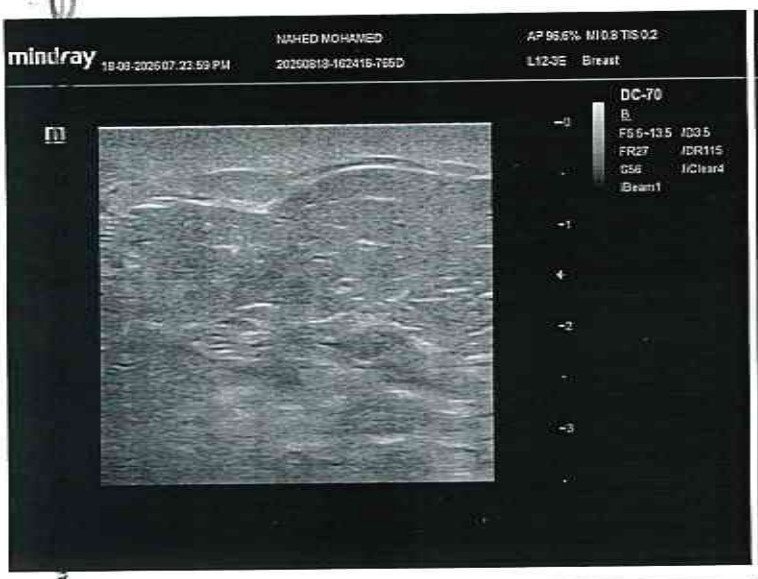
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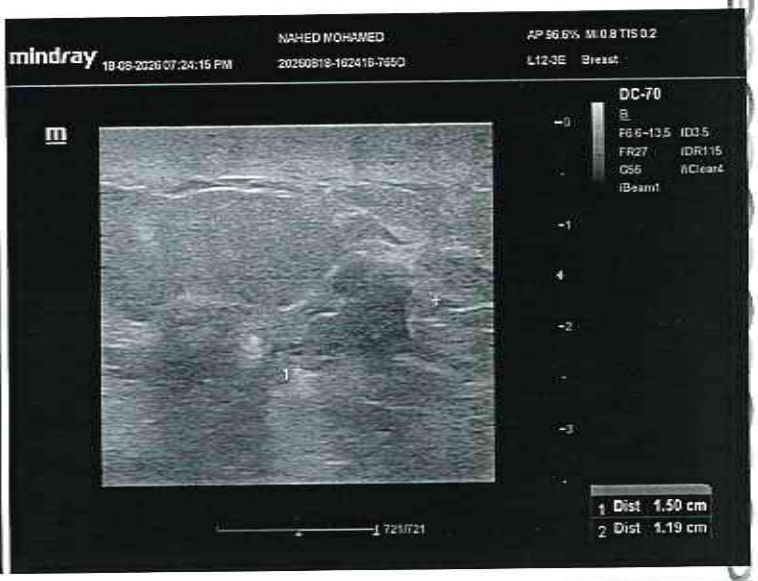
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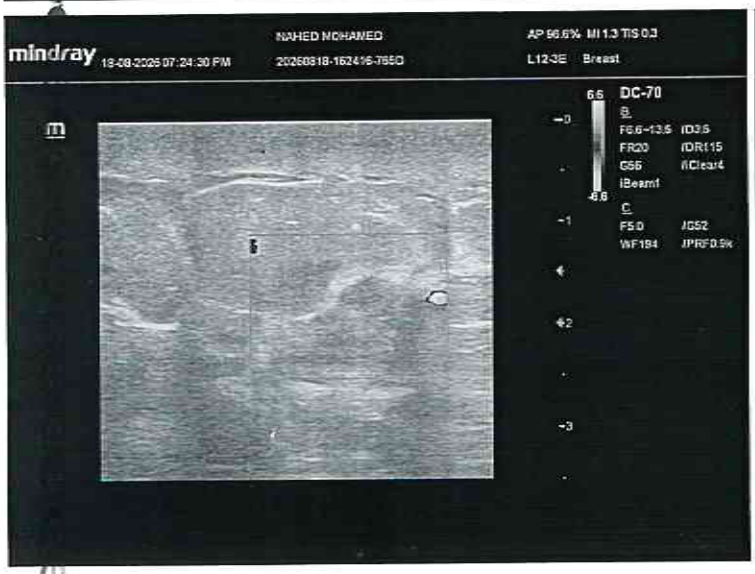
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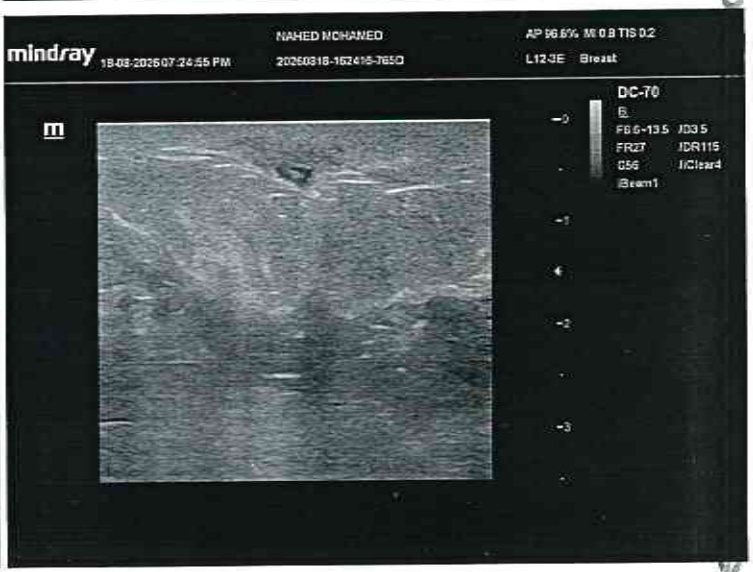
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١٨ ش اولاد عطيه - برطس
مركز أوسيم - الجيزه
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ربة منزل
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البطاقة سارية حتى ٢٠٣٣/٠٦/٢٦
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NHTMRI

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Age: 42 years old	2026/06/23 11:41:53 AM :Report Date

Bilateral Breast Sono-Mammography

CLINICAL SUMMARY: right long standing painful lump.

FINDINGS:

ACR: b (scattered fibro-glandular densities)

LEFT BREAST:

- No masses, asymmetry or distortion or suspicious microcalcifications.
- Normal skin / nipple / areolar complex.

Complementary ultrasound:

- Preserved fibroglandular tissue
- No solid or cystic lesion
- Normal nipple/areola/complex.
- No pathological axillary lymphnode

RIGHT BREAST:

- **Low energy (Non contrast) images** : UOQ & central & UIQ asymmetry with distortion & overlaying scattered calcifications extend from retroareolar to posterior third of the breast measures 10x8x7cm
- Diffuse skin thickness & nipple retraction.

Complementary ultrasound:

- 8-3:00 Diffuse pathological process in form area of altered parenchyma, mount to mass formation reaching the nipple with focal area of skin tethering & diffuse skin thickness & retracted nipple.
- Level I & II more than 3 discrete pathological nodes, the largest in level measures 14x7mm & in level II 9x5mm.

IMPRESSION:

LEFT BREAST: (BIRADS1 = normal)

RIGHT BREAST: (BIRADS 5= highly suggestive of malignancy): diffuse pathological process , highly suggestive of malignancy for US guided biopsy

Best regards,

Dr. Marwa Ramzy, MD, FRCR

(٤)

١٠٠ مليون صحة

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



لم يتم الرد
أكثر من

الست المصرية هتبقى صحة مصر

منظمة الصحة العالمية
جمهورية مصر العربية

الاسم: شاهد محمد محمد الحبيب التاريخ: ١٨/٥/٢٠٢٢
الرقم القومي: ٢٩٠٠٥١٠٢٠١٠٤٤
رقم التليفون: ٠١١٤٩١٤٦٨٧

مستهدفة بالتوعية

مستهدفة بالتوعية والفحص الإكلينيكي

الوزن	الطول	مؤشر كتلة الجسم	قياس السكر العشوائي	قياس ضغط
٩٥	١٦٥		٩٠	١٢/٨

٩٥/١٦٥/٢٠

* فكري تفسك باستمرار بموعد الفحص الذاتي شهريًا

أغسطس 2022		سبتمبر 2022		أكتوبر 2022	
نوفمبر 2022		ديسمبر 2022		يناير 2023	
فبراير 2023		مارس 2023		أبريل 2023	
مايو 2023		يونيو 2023		يوليو 2023	
أغسطس 2023		سبتمبر 2023		أكتوبر 2023	
نوفمبر 2023		ديسمبر 2023		يناير 2024	
فبراير 2024		مارس 2024		أبريل 2024	
مايو 2024		يونيو 2024		أبريل 2024	

- حافظي علي قياس السكر والضغط مرة كل ٦ شهور.
- إذا كنتي في سن الزواج أو الإنجاب، إستشيري بإنتظام طبيب تنظيم الأسرة.
- إطمني ، الكشف المبكر بيساعد في السيطرة على الأمراض، وزيادة نسب الشفاء.

موعد الفحص القادم: مستشفى أو مركز توقيع الطبيب:

المكان المحولة له للتقييم: موعد التقييم:

15335 www.100millionseha.eg

Name: NAHED MOHAMED MOHAMED	Date : 18/8/2026
Age : 31 TE : 01127872211	Exam MAMMO

Clinical presentation:

Right mastaliga, and swelling with skin changes, for assessment

MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND FOR BOTH BREASTS REVEALED:

- The right breast is the seat of diffuse increase density and skin thickening with thickening and coarsening architecture noted involving the four quadrants of right breast. Ultrasound examination showed large central glandular area of architectural distortion measured about 16x10 cm with vascularity seen para areolar involving most anterior central part of the breast reaching to subcutaneous region and nipple , some areas showed skin affection .
- Enlarged right axillary lymphadenopathy the largest 3 cm
- No micro calcification seen.
- Normal left mammogram.
- Normal skin thickness and contour of left breast.

Opinion :

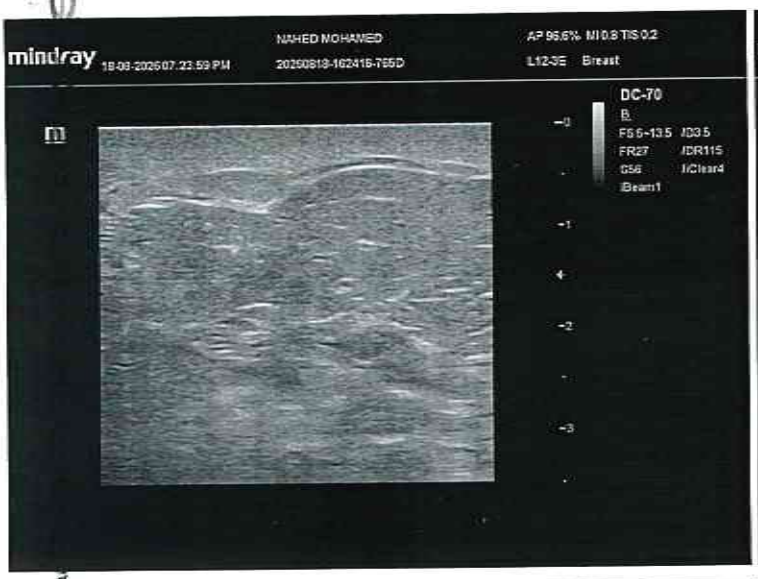
Findings are likely of right breast secondary inflammatory changes on top on central large suspicious infiltrative lesion , dynamic MRI and Biopsy recommended BIRADS 4c/5

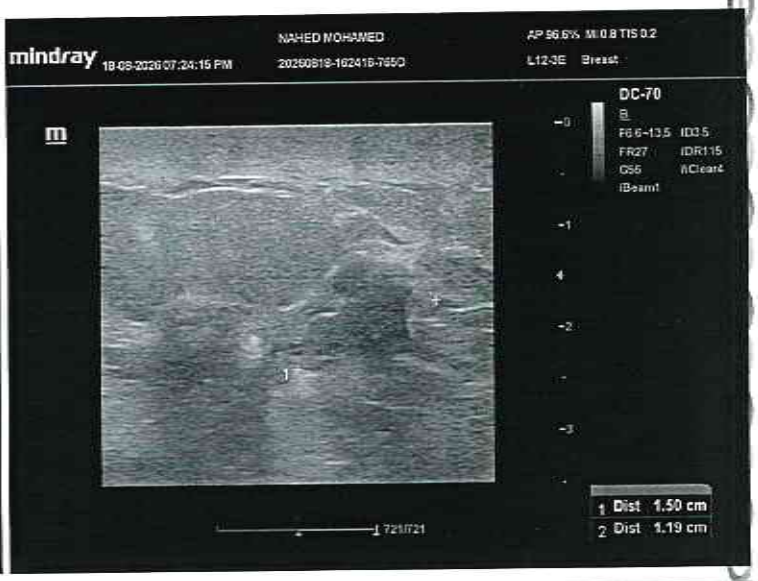
Best regards,
Dr. Eman El Taweel.MD

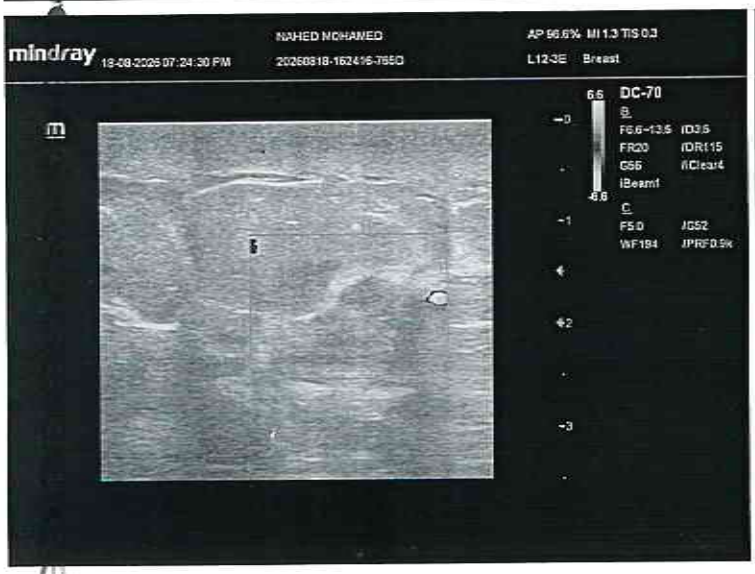
(Dear Prof doctor For Further medical questions please do not hesitate to contact me on what's up number +20 10 60442675)

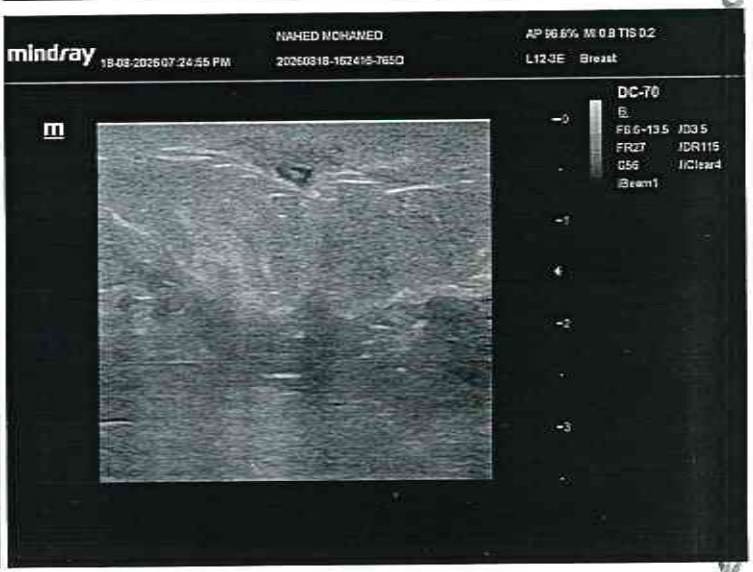
BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

- BIRADS 0 : Needs further investigations.
- BIRADS 1: Normal findings.
- BIRADS 2: Benign.
- BIRADS 3: Probably benign.
- BIRADS 4: A: low suspicious of malignancy .
- B: moderate suspicious of malignancy
- C: Highly suspicious of malignancy
- BIRADS 5: Malignant.
- BIRADS 6: Proven pathologically malignant


mindray
NAHED MOHAMED
AP 96.6% MI 0.8 TIS 0.2
L12-3E Breast
DC-70


mindray
NAHED MOHAMED
AP 96.6% MI 0.8 TIS 0.2
L12-3E Breast
DC-70
1 Dist 1.50 cm
2 Dist 1.19 cm


mindray
NAHED MOHAMED
AP 96.6% MI 1.3 TIS 0.3
L12-3E Breast
DC-70


mindray
NAHED MOHAMED
AP 96.6% MI 0.8 TIS 0.2
L12-3E Breast
DC-70


mindray
NAHED MOHAMED
20260818-162416-765D
L12-3E Breast
DC-70

[[DOCTYPE:CT_SCAN]]
CT Scan

Medical No. : 9955
Patient's Name: Nahed Mohamed Mohamed
Date: 2026-09-27
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

CLINICAL INDICATION: breast cancer workup.
COMPARISON: No previous study for comparison.
TECHNIQUE: Multiplanar CT with IV contrast CAP.

C.T CHEST

FINDINGS:
right breast mass lesion (11x5.6cm), skin thickening with right axillary rounded lymph nodes.
TRACHEA: Normal diameter and wall thickness, with no evidence of stenosis.
BRONCHI: Patent, with no evidence of bronchiectasis.
LUNGS: Multiple subcentimetric pulmonary nodules 4-6 mm scattered mainly in upper, middle and lingula.
No alveolar infiltration or interstitial thickening.
MEDIASTINUM: No mediastinal mass lesion or lymph node enlargement.
HILUM: No hilar mass lesion or lymph node enlargement.
CARDIAC CHAMBERS
No chamber dilatation.
PERICARDIUM: No pericardial thickening or effusion.
AORTA (ascending, arch and descending): Normal diameter, with no evidence of aortic aneurysm.
PULMONARY TRUNK AND MAIN PULMONARY ARTERIES: enlarged diameter 3.3cm.
PLEURA: No pleural effusion.

ABDOMEN AND PELVIS

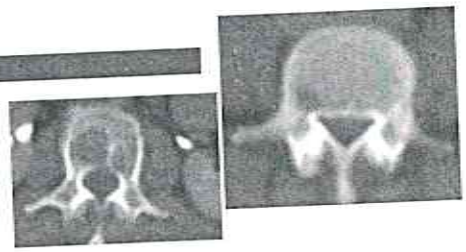
LIVER:
Size: average size
Surface: Smooth and regular.
Hepatic focal lesion: 20x17 mm
hyperattenuating enhancing focal lesion in portal phase 128HU
wash out noted in the delayed phase 83HU.
Site: subcapsular right hepatic lobe.
Another focal lesion 12x11mm same criteria in caudate lobe.
Portal vein: Patent.
Hepatic biliary system: Not dilated.
SPLEEN: **average size**. No splenic focal lesion.
KIDNEYS: Normal size, parenchymal thickness, density and enhancement. Normal contrast excretion on delayed images. No calculi or back pressure changes on either side. The perinephric fat planes are preserved.
ADRENAL GLANDS: both are average size, with no mass lesion on either side.
Pancreas: Normal size and density. No pancreatic mass lesion. The pancreatic duct is not dilated.
ABDOMINAL AORTA: Unremarkable. No aneurysmal outpouching or dissection.
IVC: Unremarkable.

[[DOCTYPE:CT_SCAN]]
CT Scan

Medical No. : 9955
Patient's Name: Nahed Mohamed Mohamed
Date: 2026-09-27
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

LYMPH NODES: No retroperitoneal, mesenteric or pelvic lymphadenopathy.
SMALL AND LARGE BOWEL: Unremarkable by CT criteria.
URINARY BLADDER: Normal wall thickness. No intravesical stone or mass.
RECTUM: Normal wall thickness. The perirectal fascia is preserved.
REPRODUCTIVE SYSTEM: unremarkable
PERITONEUM: No ascites.

SKELETON CT



L2 lytic lesion with sclerotic margin, measures about 2.1 cm.
L5 and S1 small ill-defined Lytic lesions.

IMPRESSION:

- Two hepatic focal lesions as described, likely metastatic.
- few lumbar lytic lesions as described, likely metastatic. Bone scan is recommended.
- Multiple subcentimetric lung nodules for follow up.
- Enlarged main pulmonary artery ,correlation with echocardiography is recommended.

Dr.Nada Nasser,FEBR

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر-معمل الباثولوجي

Patient Name	ناهد محمد محمد	**Case No.**	**384328**
Sex/Age	**Female 42 Y**	**Patient No.**	**26578**
Registered Date	**13/9/2026 12:57:20 PM**	**Referred By Dr.**	**Out Patient**
Validation Date	**23/9/2026 10:59:30 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**08/10/2026 09:16:54 AM**	**Pathology No.**	**SR-1009/26**

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Block revision.

Gross Examination:
One paraffin block labeled by (551/26).

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
-Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 3, pleomorphism 2, mitosis 1).
- DCIS: Absent.
- Microcalcification: Absent.
- Angioinvasion: Absent.
- Perineural invasion: Absent.
- Tumor infiltrating lymphocytes (TILs): Mild.

Results:
*** IPT (All controls showed appropriate reactivity):**
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 40% of tumor cells (score 7/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 50%.

Conclusion:
Right breast mass, Tru-cut biopsy:
-Invasive duct carcinoma, Grade 2.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).

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جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي انترناشونال

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Reporting Date	**08/10/2026 09:16:54 AM**	**Pathology No.**	**SR-1009/26**

- Her2: Positive (score 3+).
- Ki67%:50%.

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

*** Scoring system: "Her2"*
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim, MD.

[[DOCTYPE:BONE_SCAN]]
Bone Scan

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Nahed Mohammed Mohammed AbdulMajid	**Date : 14-10-2026 08:24**
Branch : Maadi	**Medical No. : 482904**
Ref.Physician Prof : Goustav Roussey Egypt	**Ins.Company : التامين الصحي**

WHOLE-BODY BONE SCINTIGRAPHY REPORT

Clinical Indication: Baseline staging in a 42-year-old female with newly diagnosed right breast carcinoma prior to surgical intervention, presenting with bony aches.

Procedure: Tc-99m MDP whole-body bone scintigraphy with anterior and posterior planar imaging.

Findings:

- Scattered focal areas of pathologically increased uptake are seen involving multiple dorsal and lumbar vertebrae.
- Multiple radiotracer-avid lesions involve few bilateral ribs.
- Discrete focal uptake is also seen at the left sacroiliac articulation and the left acetabulum.
- Focal hyperfixation is noted at the right scapula.
- The remaining long bones of the upper and lower extremities show symmetrical distribution without discrete metastatic foci.

Impression:

- Multiple scattered active osteoblastic osseous lesions involving the dorsal and lumbar spine, few bilateral ribs, right scapula, left sacroiliac joint, and left acetabulum, highly consistent with widespread skeletal metastatic disease secondary to the primary breast carcinoma, for clinical and radiological correlation and follow up as indicated.

Dr. Alaa Nagi, MD Nuclear Medicine

NILE DIAGNOSTIC CENTER